

Healthcare Logistics — Unstructured Data & Operational Stories

Expanded source material for AI PowerPoint generation. Includes narrative content, real-world reference data and clearly labelled illustrative datasets that can be converted into charts.

The supply chain behind a hospital

Healthcare logistics connects manufacturers, distributors, warehouses, hospitals, clinics, laboratories and patients. The network has to move products while preserving information about quantity, batch, expiry, temperature, destination, priority and chain of custody. WHO guidance emphasizes logistics information systems, inventory visibility, distribution planning and standardized logistics indicators as important elements of supply-chain management. [■cite■turn0search1■turn0search14■](#)

The difficult cases are often created by interactions: a supplier delay coincides with a demand spike; a vehicle becomes unavailable while a hospital receiving dock is congested; a temperature-sensitive shipment arrives late; or inventory exists in the network but not at the facility that needs it.

Real-world reference data — WHO Global Health Emergencies Logistics Hub

The following figures are published by WHO for its Global Health Emergencies Logistics Hub for the 2024–2025 biennium. They are real reported figures and can support a factual chart in a presentation. [■cite■turn0search3■](#)

Metric	WHO reported value
Operational cost savings	USD 19 million
Value of in-kind flights	USD 6.7 million
Value of supplies delivered	USD 60.2 million
Countries reached	97
Emergency charters	73
Orders delivered	1,091
Metric tons delivered	4,830
WHO business units supported	50

A second real-world data point comes from WHO's NTD supply-chain reporting. WHO states that more than 31 billion tablets and vials have been distributed globally since 2011, including almost 1.5 billion in 2025 alone. The 2026 report highlights forecasting, digital-system integration and supply-risk mitigation as ongoing priorities. [■cite■turn0search15■](#)

Chart-ready dataset: WHO emergency logistics activity

The same WHO figures can be represented as separate charts because the measures have different units. For example, countries reached, emergency charters, orders delivered and metric tons delivered are suitable for a bar chart; USD values can be shown separately.

Metric	Value	Unit
Countries reached	97	countries
Emergency charters	73	charters
Orders delivered	1091	orders
Metric tons delivered	4830	metric tons
Business units supported	50	units

A hospital network does not behave like a simple warehouse

One hospital may have 800 units of a product while another has 40. The first location can appear overstocked and the second understocked even though the total network balance is comfortable. Moving stock between locations can sometimes be faster than buying new stock, but transfer decisions need to account for expiry, transportation time, clinical priority and available substitutes.

WHO's 2025 last-mile LMIS guidance notes problems including poor data visibility, distribution planning, stock monitoring and wastage. It describes standard logistics KPIs as a way to support supply-chain decision-making. [■cite■turn0search1■](#)

Illustrative monthly performance data — created for chart testing

The following dataset is fictional and intentionally provided as presentation-test data. It can be used directly for line charts, clustered columns or before/after comparisons.

Month	On-time %	Fill rate %	Cold-chain %	Emergency shipments
Jan	91.8	93.4	97.1	184
Feb	92.6	94.1	97.5	176
Mar	93.2	94.8	97.9	169
Apr	94.1	95.0	98.1	161
May	94.8	95.7	98.4	153
Jun	95.6	96.2	98.8	137
Jul	95.1	96.0	98.6	142
Aug	96.0	96.5	98.9	131
Sep	96.4	96.8	99.1	126
Oct	96.1	97.0	99.0	124
Nov	96.8	97.3	99.2	118

Month	On-time %	Fill rate %	Cold-chain %	Emergency shipments
Dec	97.2	97.6	99.3	112

The monthly pattern can support several different visual stories. A line chart can show improvement in on-time delivery. A second line can show fill-rate movement. Emergency shipments can be plotted as a declining operational-exception series. The dataset is deliberately not tied to a real healthcare organization.

Illustrative regional comparison

Region	Orders / month	OTIF %	Avg lead time (hr)	Emergency share %	Inventory accuracy %
North Hub	4,820	96.4	6.2	5.1	98.5
South Hub	5,410	94.8	7.1	7.4	97.9
East Hub	3,960	97.1	5.8	4.6	98.8
West Hub	4,670	95.6	6.7	6.3	98.2

This dataset is fictional. It is suitable for a horizontal bar chart comparing OTIF by region, a scatter plot of lead time versus OTIF, or a grouped chart comparing emergency-order share and inventory accuracy.

Illustrative product-category demand

Category	Monthly units	Share of outbound volume	Avg expiry remaining
Surgical consumables	18,400	21%	8.4 months
Diagnostic kits	15,700	18%	10.2 months
Routine medicines	24,900	29%	14.6 months
Vaccines	7,800	9%	6.8 months
IV fluids	12,600	15%	11.3 months
Medical devices	6,900	8%	24.0 months

The category data creates a natural opportunity for a pie or stacked bar showing outbound-volume composition. It can also be used for a scatter plot where monthly volume is compared with remaining shelf life.

Cold-chain equipment and monitoring

WHO published a Cold Chain Equipment Inventory and Gap Analysis Tool in September 2025 to support recording equipment, facility segmentation, replacement planning and maintenance/running-cost estimation. WHO also published a temperature-mapping tool in January 2026 for cold-chain equipment and dry stores. [■cite■turn0search9■turn0search12■](#)

For chart testing, an illustrative facility dataset can contain equipment age, operational status, temperature excursions and maintenance cost. These numbers are fictional.

Facility	Cold rooms	Refrigerators	Equipment >8 yrs	Annual maintenance (\$K)	Excursions / quarter
Central DC	4	18	7	186	3
Hospital A	1	9	4	72	2
Hospital B	2	11	5	94	4
Hospital C	1	7	2	48	1
Regional Clinic	0	4	3	31	3

Transportation performance fragments

A route can have excellent on-time performance while still being expensive because of empty mileage. Another route may have high vehicle utilization but frequent receiving delays. This is why transportation dashboards often need more than one metric.

Quarter	Empty mileage %	Vehicle utilization %	Avg stops	Late departures %	Avg route duration hr
Q1	18.4	68	7.2	14.2	7.9
Q2	15.9	71	7.8	12.5	7.7
Q3	13.2	74	8.1	10.1	7.5
Q4	11.5	76	8.6	8.7	7.3

A scatter chart could plot vehicle utilization against empty mileage. A line chart could show the decline in empty mileage across quarters. A bar chart could compare late-departure rates.

Supplier variability

Supplier	Avg lead time days	Lead-time variability days	Fill rate %	Damage rate %	Urgent response hr
Supplier A	4.2	0.8	98.1	0.4	2.8
Supplier B	3.6	2.9	91.4	0.8	4.7
Supplier C	5.1	0.6	97.2	0.3	3.1
Supplier D	6.4	1.4	95.8	0.6	3.9
Supplier E	4.8	1.1	96.6	0.5	3.4

Fictional supplier data. This supports a scatter chart of lead-time variability versus fill rate, or a horizontal bar chart of average lead time.

Demand spike scenario

Imagine that respiratory-product demand rises over twelve weeks. The following fictional series can be used to create a line chart and test whether the PPTX system identifies the sudden acceleration rather than treating every week equally.

Week	Baseline demand	Actual demand	Emergency orders	Available stock
W1	8,200	8,050	21	12,400
W2	8,350	8,480	24	12,100
W3	8,500	8,620	26	11,900
W4	8,650	8,910	31	11,400
W5	8,800	9,380	39	10,800
W6	8,950	9,920	48	10,100
W7	9,100	10,540	61	9,200
W8	9,250	11,180	74	8,100
W9	9,400	11,760	89	7,000
W10	9,550	12,220	97	5,900
W11	9,700	12,480	103	4,800
W12	9,850	12,700	109	3,900

Inventory ageing and expiry

Expiry is another useful chart dimension. A healthcare logistics presentation can show how much inventory sits in different remaining-shelf-life bands. The following is fictional test data.

Remaining shelf life	Inventory value (\$K)	SKU count	Share of inventory value
< 1 month	42	118	3.1%
1–3 months	96	274	7.0%
3–6 months	214	512	15.6%
6–12 months	438	1,140	31.9%
12–24 months	391	1,030	28.5%
> 24 months	190	620	13.9%

This can become a stacked bar, histogram-like bar chart, or a part-to-whole visualization. It also gives the PPTX agent an opportunity to explain why short-dated inventory deserves attention.

Traceability and digital identification

WHO's 2025 report on medical-product traceability describes traceability as a mechanism for monitoring product integrity through the supply chain and supporting earlier detection and response to problems. █cite█turn0search5█ WHO's Global Health Emergencies Logistics Hub also reports piloting QR-code scanning to improve visibility of health-supply distributions. █cite█turn0search3█

A fictional digital-adoption dataset can therefore test a chart showing barcode/QR scan coverage over time.

Month	Shipment scan coverage %	Digital proof-of-delivery %	Manual exception entries
Jan	71	62	418
Feb	74	67	391
Mar	79	71	352
Apr	83	77	318
May	87	82	276
Jun	91	88	231
Jul	94	92	194
Aug	96	95	162

Sustainability and logistics

WHO's 2025 guidance on decarbonizing healthcare supply chains focuses on changing how health systems select, procure and use goods and services and on aligning sustainable procurement practices. █cite█turn0search4█ A logistics presentation can connect route efficiency, procurement choices, packaging, refrigeration and inventory waste without reducing sustainability to a single metric.

Illustrative sustainability dataset: quarterly transportation distance is 1.42M km, 1.36M km, 1.29M km and 1.21M km; empty mileage is 18.4%, 15.9%, 13.2% and 11.5%; estimated transport emissions are 1,180, 1,120, 1,050 and 980 tCO2e. These figures are fictional and intended only for chart generation.

Emergency logistics at global scale

WHO's Global Health Emergencies Logistics Hub reported USD 60.2 million in supplies delivered, 1,091 orders delivered, 73 emergency charters, 4.83 thousand metric tons delivered and 97 countries reached during the 2024–2025 biennium. The Hub also reports USD 19 million in operational cost savings. █cite█turn0search3█

These real-world figures are useful for a presentation because they provide a factual scale-of-operations slide, while the fictional datasets in this document can be used for analytical charts without implying that the fictional values are WHO measurements.

More unstructured fragments

Monday morning demand is usually moderate. Tuesday has the highest planned surgical consumption. Friday afternoon has the largest share of late orders. A hospital with the highest total volume is not necessarily the hospital with the highest emergency demand. One route has excellent on-time performance but high empty mileage. Another route has high utilization but frequent receiving delays. A supplier's average lead time improved while its variability increased. A warehouse reduced picking errors after barcode verification. A hospital reduced urgent requests after changing its internal reorder threshold. A power outage lasting 42 minutes triggered a backup generator during a previous fictional pilot.

The intended use is not to make every paragraph into a slide. The presentation-generation system should detect that some material is narrative, some is evidence, some is chart data, some is a scenario and some is supporting context. It should decide which information belongs together and construct an appropriate visual narrative.

Source notes

Real-world reference figures and statements are sourced from WHO publications and pages. Illustrative datasets are explicitly labelled as fictional and are provided for testing chart extraction, chart selection and PowerPoint layout generation.

WHO references: Global Health Emergencies Logistics Hub [\[cite:turn0search3\]](#); last-mile logistics information systems guidance [\[cite:turn0search1\]](#); cold-chain equipment inventory and gap analysis tool [\[cite:turn0search9\]](#); cold-chain temperature mapping tool [\[cite:turn0search12\]](#); medical-product traceability [\[cite:turn0search5\]](#); healthcare supply-chain decarbonization guidance [\[cite:turn0search4\]](#); good storage and distribution practices [\[cite:turn0search14\]](#).